Clinical Intake & Metabolic Screening Protocol

Subject: GLP-1/GIP Agonist Eligibility and Safety Assessment

Section 1: Personal Identification and Vital Metrics

The initial segment serves as the physiological "ground truth." These metrics define legal and clinical eligibility for pharmacological intervention.

Demographic Identification and Baseline Statistics

- **Legal Name:** [_____]
- **Date of Birth:** [MM / DD / YYYY]
- **Biological Sex:** ( ) Male ( ) Female

> **Clinical Context:** Precise age and biological sex are non-negotiable variables in calculating the estimated glomerular filtration rate (eGFR) and interpreting hormonal health risks.

Anthropometric Assessment: BMI Calculation Engine

- **Current Height:** [] Feet/Inches OR [] Centimeters
- **Current Weight:** [] Pounds OR [] Kilograms
- **Calculated BMI:** [Auto-populated: $BMI = \frac{weight(kg)}{height(m)^2}$]

> **Clinical Context:** FDA-approved labeling for Wegovy and Zepbound defines eligibility as $BMI \geq 30kg/m^2$ (Obese) or $\geq 27kg/m^2$ (Overweight) with comorbidities. Data must be recorded within the last 30–90 days for insurance authorization.

Hemodynamic Baseline Monitoring

- **Blood Pressure:** [] / [] mmHg
- **Resting Heart Rate:** [____] BPM

> **Clinical Context:** GLP-1 RAs increase resting heart rate by 2–4 BPM. Baseline BP is a primary qualifying comorbidity for patients in the $27–29.9$ BMI range.

Reference: BMI Clinical Designation

BMI Category	Range (kg/m²)	Clinical Designation	Coverage Implications
Underweight	< 18.5	High Risk	Generally contraindicated.
Healthy Weight	18.5 – 24.9	Normal	Rarely eligible (except maintenance).

Overweight	25.0 – 29.9	Pre-Obesity	Eligible if comorbidities (HTN, T2D) present.
Obese (Class I)	30.0 – 34.9	Clinical Obesity	Meets primary criteria for medication.
Obese (Class II+)	> 35.0	Severe Obesity	High priority for intervention.

Section 2: High-Risk Clinical Screening and Oncology Vigilance

Absolute contraindications screened with zero tolerance for ambiguity.

Thyroid C-Cell Neoplasia and Genetic Syndromes

- Do you or any first-degree relative (parent, sibling, child) have a history of Medullary Thyroid Carcinoma (MTC) or Multiple Endocrine Neoplasia syndrome type 2 (MEN 2)?
  - ( ) Yes ( ) No

  Clinical Context: The GLP-1 class carries an FDA "Boxed Warning" for thyroid C-cell tumors. History of MTC or MEN 2 is an absolute contraindication.

Pancreatic and Biliary Disease

- Have you ever been diagnosed with Pancreatitis? ( ) Yes ( ) No
- Do you have a history of Gallstones or Gallbladder removal? ( ) Yes ( ) No

  Clinical Context: Identifying baseline gallbladder disease allows monitoring for acute cholecystitis as weight loss accelerates. History of pancreatitis carries high risk for re-inflammation.

Gastric Motility: Silent Gastroparesis

- Do you experience frequent nausea, early fullness, or have you been diagnosed with Gastroparesis?
  - ( ) Yes ( ) No

  Clinical Context: GLP-1s slow gastric emptying. In patients with pre-existing delayed emptying, this can lead to bowel obstruction or intractable vomiting.

Section 3: Organ Systems Review and Laboratory Integration

Baseline assessment to differentiate pre-existing pathology from potential treatment side effects.

Renal Function and Nephroprotection

- History of Kidney Disease? ( ) Yes ( ) No
- **Recent Labs (Optional):** eGFR: [*] | *Creatinine: [*]

Clinical Context: Therapy is used with extreme caution or contraindicated when eGFR falls below $30mL/min/1.73m^2$.

Ocular Health: The Retinopathy Paradox

- If Diabetic: Have you been diagnosed with Diabetic Retinopathy? ( ) Yes ( ) No
- Date of last Dilated Eye Exam: [MM / DD / YYYY]

Clinical Context: Rapid HbA1c improvement can paradoxically worsen retinopathy ("normoglycemic re-entry"). Documentation within the last 12 months is required.

Cardiovascular Assessment

- Check if you have a history of: [ ] Heart Attack [ ] Stroke [ ] Heart Failure

Clinical Context: GLP-1s (SELECT/LEADER trials) reduce MACE (Major Adverse Cardiovascular Events). Identification helps justify "medical necessity."

Lab Metric	Normal Range	Clinical Significance
HbA1c	< 5.7%	Baselines glycemic control; identifies T2D.
eGFR	> 90 mL/min	Assesses renal safety and hydration.
TSH	0.4 – 4.0 mU/L	Rules out hypothyroidism as cause of weight gain.

Section 4: Current Medication and Substance Review

Drug-drug interaction screening based on glucose lowering and gastric emptying dynamics.

Iatrogenic Hypoglycemia Risk

- Are you taking Insulin or Sulfonylureas (e.g., Glipizide, Glyburide)? ( ) Yes ( ) No
- **If Yes, Current Dose:** [_____]

Clinical Context: GLP-1s are glucose-dependent, but combined with insulin/sulfonylureas, they can cause life-threatening hypoglycemia.

Oral Medication Absorption

- Do you take "narrow therapeutic window" drugs (e.g., Warfarin, Digoxin, Lithium, Levothyroxine)?
  - [Search/Select Medication]

Clinical Context: Delayed gastric emptying can alter the T_{max} and C_{max} of oral medications, potentially leading to sub-therapeutic levels.

Section 5: Longitudinal GLP-1 History and Titration Dynamics

Historical Dosing and Tolerability

- **Medications Used:** [ ] Ozempic [ ] Wegovy [ ] Mounjaro [ ] Zepbound
- **Highest Dose Reached:** [_____]
- **Date of Last Dose:** [MM / DD / YYYY]

> **Clinical Context:** If a patient has been off medication for > 14 days, tolerance has reset; resuming at high doses requires hospitalization for severe vomiting.

Side Effect Profiling

- During previous use, did you experience:
  - [ ] Nausea [ ] Vomiting [ ] Diarrhea [ ] Constipation [ ] Reflux
  - **Severity (1-10):** [____]

> **Clinical Context:** Pre-emptive anti-emetic prescriptions (Zofran) improve long-term adherence.

Section 6: Psychological Readiness (SCOFF Questionnaire)

1. Do you make yourself **S**ick because you feel uncomfortably full? ( ) Yes ( ) No
2. Do you worry you have lost **C**ontrol over how much you eat? ( ) Yes ( ) No
3. Have you recently lost more than **O**ne stone (14 lbs) in 3 months? ( ) Yes ( ) No
4. Do you believe yourself to be **F**at when others say you are thin? ( ) Yes ( ) No
5. Would you say that **F**ood dominates your life? ( ) Yes ( ) No

- **Motivation for Treatment:** [ ] Health [ ] Appearance [ ] Mobility
- **Goal Weight:** [_____]

> **Clinical Context:** SCOFF is sensitive for Anorexia/Bulimia. GLP-1s reduce "food noise" but can lead to anhedonia in patients with mood instability.

Section 7: Reproductive Health and Teratogenicity Safety

- Are you pregnant, breastfeeding, or planning to conceive? ( ) Yes ( ) No

> **Clinical Context:** A "washout" period of at least **2 months** is mandated before conception due to the long half-life of these medications. Fetal growth abnormalities have been observed in animal studies.

Clinician Signature: _____ **Date:** _____